

CPR Debriefing Report

Simulation Lab · 2026-05-27

Session SES-SCN-002 · Adult VF Cardiac Arrest with Multiple Workflow Errors and ROSC

Team Leader: Simulation Team

Guideline: AHA_2020

1. Scenario summary

Session ID	SES-SCN-002
Date	2026-05-27
Scenario	Adult VF Cardiac Arrest with Multiple Workflow Errors and ROSC
Scenario type	VF
Team leader	Simulation Team
Team size	6
Duration	14m 00s
Guideline	AHA_2020

Overall score	Grade	Findings	Critical	High	Moderate
100 / 100	A	0	0	0	0

2. Domain scores

Scores are derived from AHA 2020 guideline rules applied to SimMan 3G sensor data and audio transcripts. Confidence indicators reflect data completeness — scores marked medium or low confidence should not be over-interpreted.

Domain	Score	Confidence	Key finding
Cpr Quality	30/100 CI: 10–30	Low_Data confidence	—
Shock Delivery	15/100 CI: 8–15	Partial_Data confidence	—
Drug Administration	15/100 CI: 6–15	Low_Data confidence	—
Rhythm Recognition	10/100 CI: 1–10	Low_Data confidence	—
Reversible Causes (H'S & T'S)	10/100 CI: 0–10	Low_Data confidence	—
Team Leadership	10/100 CI: 0–10	Low_Data confidence	—

Team Communication	10/100 CI: 0–10	Low_Data confidence	—
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3. Protocol deviations

CRITICAL

R_ARREST_001

@ 5:08

CPR delayed 308s after arrest recognition (limit 30s)

Guideline reference: AHA 2025 Adult Cardiac Arrest Algorithm
Finding ID: fnd_0002

HIGH

R_ARREST_011

@ 0:00

Neither amiodarone nor lidocaine given after 3 or more shocks in refractory shockable rhythm

Guideline reference: AHA 2025 ACLS Algorithm — Antiarrhythmic After 3rd Shock
Finding ID: fnd_0004

HIGH

R_ARREST_016

@ 0:00

12-lead ECG not documented after ROSC

Guideline reference: AHA 2025 Post Cardiac Arrest Care
Finding ID: fnd_0005

HIGH

R_ARREST_024

@ 0:00

Basic airway management not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Airway
Finding ID: fnd_0007

HIGH

R_ARREST_025

@ 0:00

Bag mask ventilation not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Breathing
Finding ID: fnd_0008

HIGH

R_ARREST_026

@ 0:00

Defibrillator attachment not documented

Guideline reference: AHA 2025 Primary ABCD Survey — Defibrillation
Finding ID: fnd_0009

HIGH

R_ARREST_028

@ 0:00

IV or IO access not documented during resuscitation

Guideline reference: AHA 2025 Secondary ABCD Survey — Circulation
Finding ID: fnd_0011

MODERATE

R_ARREST_027

@ 0:00

Advanced airway not placed during resuscitation

Guideline reference: AHA 2025 Secondary ABCD Survey — Airway
Finding ID: fnd_0010

INFO

R_ARREST_005

@ 4:10

Rhythm check interval 195s exceeds 120s (+15s grace)

Guideline reference: AHA 2025 Adult Cardiac Arrest Algorithm
Finding ID: fnd_0001

INFO

R_ARREST_005

@ 9:05

Rhythm check interval 295s exceeds 120s (+15s grace)

Guideline reference: AHA 2025 Adult Cardiac Arrest Algorithm
Finding ID: fnd_0003

INFO

R_ARREST_017

@ 0:00

Temperature management not documented after ROSC

Guideline reference: AHA 2025 Post Cardiac Arrest Care
Finding ID: fnd_0006

4. Communication analysis

Detailed communication analysis requires audio transcription. Ensure lapel and ceiling mic audio files are provided for full closed-loop communication metrics.

7. Strengths

No strengths data available from narrative synthesis.

5. Reflective prompts

The following questions are mapped to specific findings detected during this session. Use them to guide the debriefing conversation with the team leader.

1. How could the team improve timing and execution for this step?
2. How could the team improve timing and execution for this step?
3. How could the team improve timing and execution for this step?
4. How could the team improve timing and execution for this step?
5. How could the team improve timing and execution for this step?
6. How could the team improve timing and execution for this step?

6. Recommendations

Ranked by clinical impact. Address critical and high-severity findings before moderate ones in the next training session.

1. Begin chest compressions immediately — within 30s of arrest recognition.
2. For VF/pVT refractory to 3 or more shocks: administer amiodarone 300mg IV/IO (repeat 150mg once) OR lidocaine 1-1.5 mg/kg IV/IO as alternative.
3. Always obtain 12-lead ECG immediately after ROSC to screen for STEMI.
4. Document airway opening technique used during resuscitation.
5. Document ventilation method used. Bag mask ventilation should begin immediately after airway opened.
6. Always document defibrillator attachment time for audit purposes.
7. IV or IO access is required for drug administration. Document route used.
8. Consider advanced airway placement for prolonged resuscitation to enable continuous compressions.